

To introduce myself, Joan G. Woodward, age 58, consulting engineer with Primary Progressive Multiple Sclerosis.

In the beginning of my “cellular” journey I was 33 years old having been married 5 years with a history of four(4) first tri-mester miscarriages. This Duke University Engineering school graduate with a Professional Engineering license was not about to give up and live with the diagnosis-“you will never be able to have biological children.” After making an appointment with a well renowned In-vitro fertilization clinic, a young Doctor newly relocated from the Miami, Florida area suggested that I might qualify for a clinical trial supervised by Marie-Louise Lubbs, PhD. She was doing research on Lymphocyte Immune Therapy (LIT).

“Lymphocyte Immunization Therapy or LIT is a procedure whereby white blood cells from the prospective father are injected into the skin of the prospective mother to prepare the maternal immune system for pregnancy. Because pregnancy tissues are the product of both the mothers as well as the fathers genes, LIT assists the mother’s immune system in the development of immunologic tolerance to the genetically foreign pregnancy tissues.”

Apparently, my body was not recognizing the pregnancies as such and my immune system was literally killing the embryos. Similar to immunizing our children against known diseases, the LIT therapy consisted of two treatments 30 days apart. Today I have a 24 year old daughter and a 22 year old son thanks to the efforts of science and the medical community. My procedures did not involve drugs and both my husband and I were extensively screened for all diseases prior to immunizing my arms with his white blood cells. We signed all waivers after having extensively researched the facility and it’s Doctors.

Jump forward to January 30, 2002 the FDA brought all clinics offering this treatment to a halt:

January 30, 2002

Dear Colleague:

It has come to the attention of the United States Food and Drug Administration (FDA) that you may be administering allogeneic human cells as a therapy for recurrent miscarriage. This treatment is often referred to as lymphocyte immune therapy (LIT). We want to advise you that the FDA has jurisdiction over such cellular products, and to inform you regarding the FDA regulatory process governing such products. These products are regulated as biological products subject to licensing under Section 351 of the Public Health Service Act (PHS Act). These products also meet the definition of “drug” and are subject to the requirements of the Federal Food, Drug, and Cosmetic Act (FD&C Act). Therefore, until such time as there is an approved Biologics License Application for the product, administration of such cells or cellular products in humans can only be performed as part of clinical investigations, and then only if there is an Investigational New Drug application (IND) in effect.

My father and I share A- blood. During a procedure in Winter Park, Florida the hospital stock piled my blood in case an infusion was necessary. The hospital did not require an IND to use blood products for this. What is the difference between the two?

Alan Beer practiced LIT until the publication of this letter in 2002. Upon cessation of LIT at his center, a significant increase in pregnancy loss was noted amongst patients being seen for recurrent pregnancy

loss (16% to 36%). However, today doctors in the USA are unable to provide this therapy. Many patients, however, still wish to seek LIT in other countries where the therapy remains available.

Jump forward to May 6, 2014. After limping for over a dozen years and being treated for a possible hip replacement, I once again completed additional research and found a new orthopedic surgeon that referred me to Emory Neurology here in Atlanta, GA. I had a single lesion which appeared on my brain stem MRI. It took until September 2, 2014 for a second lesion to appear. I am in excellent health and have never had a so-called “attack”. Still, the dreaded words “You have Primary Progressive Multiple Sclerosis”.

Since then I have spent at least an hour a day researching this disease, not for one minute accepting the dreadful diagnosis of no relief in sight. I have joined a clinical trial for a new drug and have exhausted nearly all avenues. Multiple MRI’s have shown my disease to be progressing. The clinical trial drug is a double blind so I deducted that either I was receiving the placebo, or the drug was not helping my symptoms. Therefore, this past week my family and I elected for me to receive mesenchymal stem cells harvested from my own adipose tissue. Literally, my fat may save me!

After carefully researching several clinics, protocols and Doctors, I chose Stemgenex as my provider. My cells, my blood, and my decision. How could this possibly be considered a New Investigational Drug (NID) ? Every individual I spoke to that received this therapy was well informed and had completed the same amount of research. At no point have I been promised results, or a cure. Already I have stopped taking an extremely expensive drug for fatigue. That in itself is a huge plus.

Newspaper articles and MS forums have now published information regarding a meeting with the FDA on April 13, 2016 in Maryland. The Boston Globe claims that :

“But the Food and Drug Administration recently issued [draft guidelines](#) clarifying that the stem cells used in most clinics are drugs and require a rigorous approval process before they can be used in patients. A [public hearing](#) is set for April.”

I sincerely hope and pray that this does not result in others not having the opportunities that I have had.

The FDA has it’s hands full with regulating new investigational drugs. Their efforts are greatly appreciated by the general public. So let’s keep their efforts where they belong, studying drugs, not the cells that God gave us.